

Experiences of first-generation immigrant parents accessing early services for their autistic children in the UK[☆]

Gamze Kaplan^{a,*,1} , Paulette Luff^{b,2}

^a Division of Psychology & Mental Health, The University of Manchester, UK

^b School of Education, Faculty of Arts, Humanities, Education and Social Sciences, Anglia Ruskin University, UK

ARTICLE INFO

Keywords:

Immigrant parents
Cultural diversity
Minority
Intersectionality
Autism

ABSTRACT

This study aims to understand the experiences of immigrant parents who have autistic children in accessing early services from the pre-diagnosis period to education. The participants of this research involve eight first-generation immigrant parents who migrated from Türkiye to the UK and have an autistic child(ren). Research data were collected using semi-structured interviews and supported by the researcher's reflective journals and documents involving medical records and Education Health and Care Plans. The data were analysed using the interpretative phenomenological analysis approach. The findings indicated that language barriers and cultural patterns of perceptual and interpersonal communication affect parental adaptation and access to early services in both positive and negative ways, with each experience reflecting a unique path. These findings have implications for policymakers and practitioners to ensure that early intervention and support services are accessible to all children and families, irrespective of their cultural background and language barriers.

1. Introduction

Autism is a neurodevelopmental condition that is typically identifiable by the age of three, though unique neurodivergent features can be evident from early infancy (Webb & Jones, 2009). It is characterised by differences in social interaction, communication styles, sensory processing, and cognitive behaviours (APA, 2013). While the journey of getting a diagnosis to access available services can vary for each family, it may begin with the observation of the child within the family or immediate environment. Upon recognising autistic features, families typically reach out to healthcare professionals such as general practitioners (GPs), health visitors, or any other professional like Special Educational Needs Coordinators (SENCOs), to seek referrals or collaboration (NHS, 2019a). This stage is shaped by the interactions of the parents, healthcare professionals and educational professionals. Later, the child is referred to the assessment process, and it is necessary to continue the cooperation in this process and contribute to the reliability of the assessment by all experts who know the child. Upon the identification of the child's condition, various support systems become available, including in-school support, social care, financial assistance, and well-being services offered by government bodies, local authorities, and charitable organisations (National Autistic Society, 2020). Families are expected to access these support systems, as recommended by

[☆] This study is part of a master's thesis and has been funded by the Republic of Türkiye Ministry of National Education.

^{*} Correspondence to: Jean McFarlane Building, 176 Oxford Rd, Manchester M13 9PY, UK.

E-mail address: gamze.kaplan@manchester.ac.uk (G. Kaplan).

¹ <https://orcid.org/0000-0003-4613-1359>

² <https://orcid.org/0000-0002-5211-1162>

the NHS (2019b).

While the system expects parents to seek available services, they may experience various psychological reactions as described by Kübler-Ross's stages of grief (2014): denial, anger, bargaining, depression, and acceptance. These stages do not necessarily need to be followed in a specific order, and experiencing one stage does not preclude experiencing it again. Therefore, understanding the psychological processes experienced by families to ensure organised support within the early environment and support systems is crucial. The Children Act, (2004) mandates assessment, coordination, and service planning for the welfare of the child and family, emphasising the importance of a holistic approach (Article 9A, 10, Section 17). Nevertheless, parents of autistic children often experience stress (Li et al., 2022) and lowered quality of life (Feng et al., 2022) due to delays/ long waiting lists and when they reach the services, a lack of consistency and inadequacy in support services (Marsack-Topolewski & Weisz, 2020).

In addition to navigating challenges in accessing early support services and education, being an immigrant adds an extra layer to their experience, and this intersectionality can compound experiences of marginalization and systemic oppression, influenced by both their ethnic identity and the advocacy required for a neurodiverse child. A recent review by Kaplan & Celik (2023) on parents from minority ethnic backgrounds raising children with disabilities highlights how cultural frameworks shape how disability is interpreted; sometimes through notions of spiritual testing, shame, or temporary delay rather than as a long-term condition. These interpretations intersect with structural factors, such as limited knowledge of available services; language barriers during clinical and educational encounters; and economic difficulties tied to migration. Migration often disrupts extended family networks, reducing access to informal caregiving support, particularly from grandparents. For example, while around 80 % of grandparents are involved with the care of their grandchildren in the UK (Information NOW, 2024); however, immigrant parents may be away from that grandparent support due to distances and visa requirements, and they may need to take this responsibility on their own, while also trying to build new relationships with people around (Souralova, 2020). Together, these dynamics reveal how cultural meaning-making is not isolated from but embedded within systemic inequalities.

Although there has been an increase in studies exploring the intersectionality of cultural and linguistic diversity with a disability, the experiences of immigrant parents in England, particularly in relation to neurodiversity, have yet to be fully understood. As one of the ethnic minority groups in England, research on Turkish parents and their children with disability is very limited (Sandhu et al., 2017). The migration experience of Turkish-speaking people started with Cypriot Turks in the 1940s and then continued with the migration of Turkish and Kurdish communities (Uysal, 2016). Currently, the number of Turkish-speaking people has reached approximately 500,000 (House of Commons, 2011). In this study, the term "Turkish parents" is used to describe parents who are from Türkiye and have migrated to England, regardless of their ethnicity including Turkish, Kurdish or Zaza origin. Despite being a prominent group, Turkish children in the English education system have one of the lowest success rates (Baykusoglu, 2014).

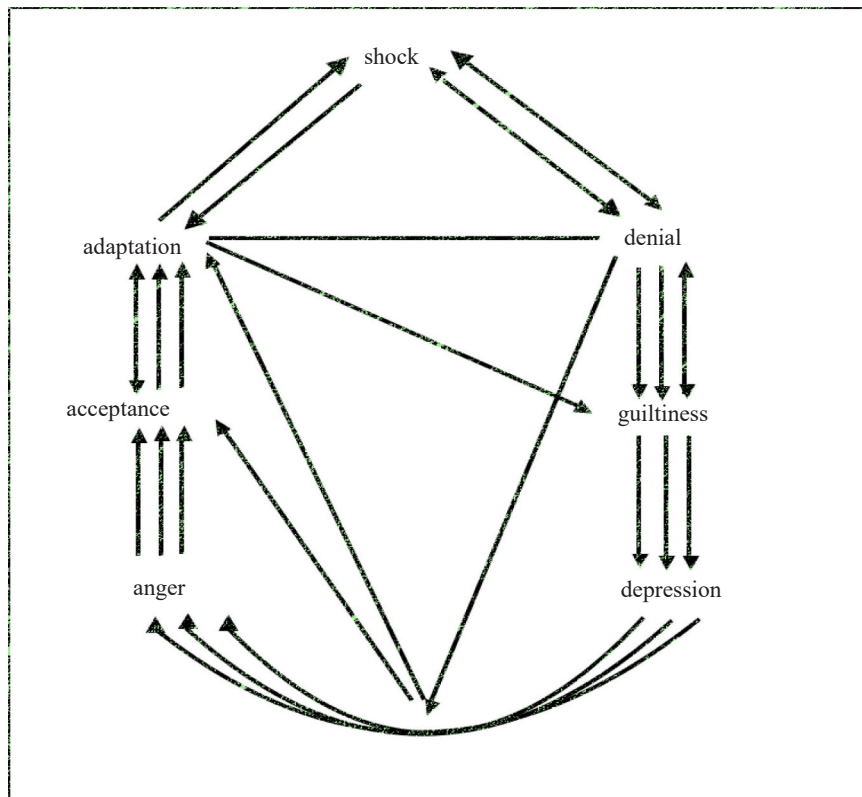


Fig. 1. Kübler-Ross's stages of grief from Varol (2006).

Additionally, Turkish children who are second-language learners often face suspensions in special education referrals. This could be attributed to confusion between language learning challenges and developmental conditions, leading to delays or incorrect assessments. (Essex County Council, 2011). Given these challenges and the lack of research, it is crucial to understand the experiences of this overlooked group to better address their needs within early support services.

The aim of this study was to address this gap by exploring the experiences of first-generation immigrant Turkish parents with autistic children in accessing early support services from the pre-diagnosis stage through the education process.

2. Materials and methods

2.1. Research design

This study was designed with a phenomenological research methodology, which is an approach to exploring an individual’s life experiences and obtaining a deep understanding of how they make sense of their experiences (Taylor et al., 2016) by focusing on the lived experiences of immigrant Turkish parents.

The debate is currently ongoing regarding the definition of phenomenology and which studies are phenomenological. While Spiers and Smith (2019) defined such studies as those that attempt to understand how their participants make sense of their experiences, van Manen (2017) argued that the experience itself reveals the reality of the experience, not how it is perceived by the participant. Meanwhile, Zahavi (2019) claimed that whether or not a piece of research is phenomenological is related to the role of the researcher.

The difference between these definitions lies in how each approach conceptualises the study of lived experience. Descriptive or transcendental phenomenology, as articulated by Husserl (1931), seeks to identify the essential structures of consciousness with approaching phenomena from a more detached, epistemological stance. In contrast, Heidegger’s (1962) hermeneutic phenomenology, emphasises understanding as an interpretive and embodied mode of being. For Heidegger, Dasein is always already situated in a world, and its understanding arises not from abstract reflection but from lived, embodied engagement with its environment. The researcher, as a being-in-the-world, cannot bracket off their context but instead brings a pre-understanding shaped by their own history and experience. This study therefore adopts a hermeneutic phenomenological approach, recognising the interpretive potential of the researcher’s shared cultural, linguistic, and embodied experience as an immigrant. Rather than a source of bias, it is understood as a meaningful ground for interpreting the lived experiences of participants.

Yin (2013) described the use of a well-known research technique as one of the ways to ensure trustworthiness. The researcher previously participated in post-graduate lectures on scientific research methods and qualitative research methods and had experience with qualitative research conducted through document analysis but had no experience using interviews as the main data collection method. To eliminate this limitation, regular meetings with the supervisor were conducted, expert opinions on the development of the interview questions were consulted, and at each step of the analysis, cooperation proceeded with the supervisor.

2.2. Participants

Parents were eligible for the study if they (a) were first-generation immigrants born in Türkiye and (b) had an autistic child who had been diagnosed in England to understand the intersection of the experiences of migration and having an autistic child. Eight parents who agreed to participate in the study were recruited via Turkish family social media groups and snowballing. An exemption was made for a mother, who was born in England, was recruited because she was willing to participate as the partner of the father participant and had shared experiences with him.

Table 1 provided some descriptive information about each of the participant parents, who consist of seven mothers and one father. The families migrated to England after 2005 and now reside in various cities.

2.3. Data collection

The research started after the approval of the [name of the university was deleted for blind review] School of Education and Social Care Research Ethics Panel [reference number deleted for blind review]. The study used semi-structured interviews as the primary data

Table 1
Participant information.

No	Age of			Educational status	Monthly income	Regions	Year of the arrival UK	Sibling
	parent	child	diagnosis					
1	34	7	3	Bachelor	2000–2500	London	2006	1
2	42	14		Bachelor	1000–1500	North West	2005	3
3	41	6		High school			Born in England	
4	40	5	3	High school	2000–2500	East Midland	2007	1
5	28	3	2	Bachelor	1500–2000	London	2012	0
6	33	7	3	Bachelor	2000–2500	London	2010	1
7	39	9	5	Bachelor	2000–2500	London	2006	1
8	31	7	3	High school	-	London	2007	2

collection technique, supported by the researcher's reflective journals and documents involving medical records and Education, Health and Care Plans (EHCP), which were voluntarily shared by four parents. The researcher prepared the topic guide, and three different expert views were taken from experienced qualitative researchers. A pilot interview was conducted to finalise the topic guide (final version presented in Table 2). The informed consent was obtained both in written and verbal form, and participants were informed about anonymisation, their option to pause or stop the interview, and their right to withdraw the data. The interviews were conducted and transcribed with anonymisation by the primary researcher. Participants were given the option to choose between speaking Turkish or English, all preferred Turkish. The interviews were conducted over Zoom, lasting between 42 and 82 min, with permission for recordings granted.

During the interviews, parents had no issues disclosing their experiences and feelings, and they shared sensitive moments, such as crying. To manage emotional intensity, participants were reminded from time to time that they could take a break at any time.

2.4. Data analysis

In this study, Interpretative Phenomenological Analysis (IPA) was used, which is a strongly idiographic approach that focuses on detailed analysis of the case either as an end in itself or before moving to similarly detailed analyses of other cases (Smith et al., 2009). The analysis involved several steps, such as meaning-making, uncovering emergent themes, creating individual and group tables of themes, and writing up (Spiers & Smith, 2019). Each interview was analysed separately and completed before moving on to the next, ensuring an idiographic commitment to the data (*ibid*). The emergent themes were produced by further evaluation and cross-examination of similarities and differences among the sub-codes. Finally, the parent superordinate themes were identified to address the research questions.

Once the interpretive themes were identified, the findings were then examined through a conceptual lens. Regarding the use of theory in phenomenological research, van Manen (2014) states that theory may be applied when it meaningfully intersects with research findings. In this study, Bronfenbrenner's (2006) bioecological theory was brought in after the analysis, as the themes pointed to different layers of influence that is ranging from immediate environments like family and services to broader social and cultural contexts. The theory provided a useful structure for presenting and situating the findings, without shaping the initial process of interpretation.

3. Theory

Bronfenbrenner's ecological theory (1979) emphasises that a child's development is not only influenced by the child but also by the systems around them. They proposed four subsystems that are closely and distantly connected to the child and highlighted the importance of understanding the relationships between these systems. These subsystems are: (a) microsystem, the immediate environment where a child spends regular time, such as parents, teachers, and friends; (b) mesosystem, the relationship between two or more microsystems (e.g., family experiences affecting school relations); (c) exosystem, the external environment that is not directly related to the child, but the decisions taken and events occurring there affect the child and their microsystems such as educational policies; and (d) macrosystem, the broader cultural context. The theory was later updated to *bioecological systems theory*, which includes biological factors and the "chronosystem," which emphasises the interaction between ecological systems over time (Bronfenbrenner & Morris, 2006).

4. Findings and interpretation

The findings were presented within the framework of Bronfenbrenner's (2006) bio-ecological systems and discussed in reference to the relevant literature and within the research scope. The themes identified were given in Fig. 2 and organised according to the different ecological systems; however, the primary emphasis was placed on the microsystem and mesosystem levels, as these layers most directly impact the daily experiences of the child and the parents.

Table 2
Semi-structured interview topic guide.

-
- Could you start by telling us a bit about yourself and your child, so we can get to know you better?
 - At what point did you begin to notice any differences in your child's development, and what signs stood out to you?
 - Can you describe the journey leading up to your child's diagnosis of special needs?
 - What was your experience like during your child's diagnostic process?
 - After receiving the official referral, could you share your experience with the decision-making process around your child's education?
 - What has your experience been like throughout your child's educational journey?
 - Looking back on the entire process, what were your expectations of the people or institutions involved? Were those expectations met?
 - In your view, how has cultural diversity influenced your experience during this process?
 - How has having a child with special needs affected your daily life, including family dynamics, social relationships, and work life?
 - What advice or suggestions would you give to other families who are just beginning this journey?
 - Is there anything else you would like to share with us?
-

* The term "special needs" was deliberately used during the interviews to align with the terminology commonly used within the community and to minimise potential disruption to the natural flow of conversation.

4.1. Micro-system – child-related findings - experiencing the journey of recognition

Although this study focused on parental experiences, the child was positioned at the core of the bio-ecological systems theory, with parents considered part of the microsystem, as all findings were reflected through the parental perspective with a focus on the child. This section presents findings related to the child, particularly their early and accompanying features, which formed the basis for accessing early support services.

4.1.1. Initial features

Parents reported early features of autism in their children, such as a lack of response to their names, reduced eye contact, repetitive behaviours/stims, and delayed motor development. One mother noted: *"When I called my daughter's name, there was no reaction. She stopped making eye contact, and repetitive behaviours became frequent."* Another mother described her son: *"He stopped listening and responding to us, not using his pointing finger, as if he was shutting down into his own world."*

Parents described a range of accompanying features they observed in their children during the early years. One mother encapsulated the sleep issues by stating, *"The biggest problems of these children are both not being able to sleep at night and waking up at night"*. Hyperactivity was also noted by several participants. One parent described their child's behaviour as *"The hard thing is not that he is autistic, but that he is hyperactive with autism. He doesn't stop for 5 s."* Moreover, the children reportedly had eating issues such as sensitivity to textures, portion control challenges, and aversions to certain types of food. For example, a mother said, *"They don't like juicy foods very much."*, illustrating a reluctance to eat foods with certain textures, a common manifestation of sensory-based food selectivity.

4.2. Microsystem - parent-related findings - acceptance, resilience and growth through adversity

In addition to the child-related findings, parent-related findings have been located in the microsystem, and the findings were related to the psychological experiences of parents presented in this section, including recognition, acceptance, research, and life experiences.

4.2.1. Recognising early developmental features

Parents recognised early signs of autism at varying points, ranging from two months of age to school age. The differences in the child were primarily noticed by the mother, although sometimes aunts, grandmothers, or parents of other children may also observe them. For instance, a father recounted, *"My sister told me. When I went to Türkiye, my sister went to a doctor there and made an appointment"*, while a mother explained, *"I told my mother about my suspicions regarding [child]'s autism. My mother agreed, saying, 'I think*

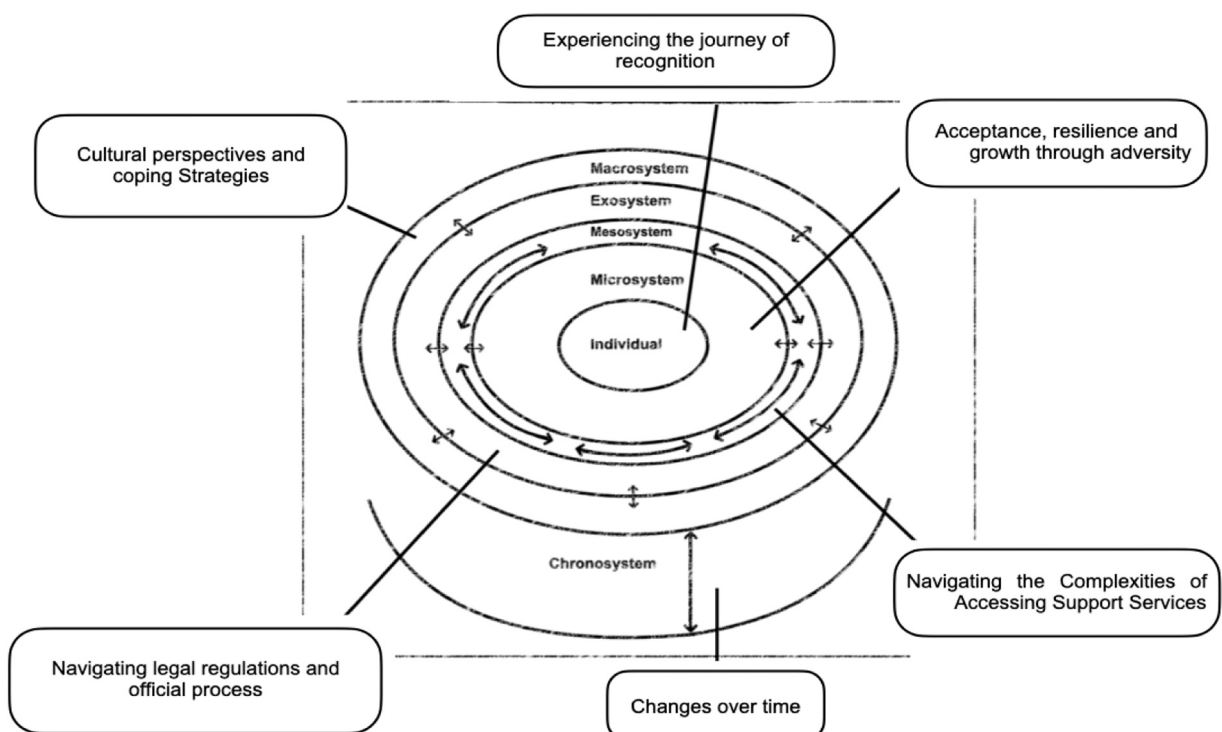


Fig. 2. The findings within the framework of the bio-ecological systems theory.

so too'.", similar to findings by [Sicherman et al. \(2018\)](#), where grandmothers were often the first to notice features. However, variations exist, as one mother described, *"We are still discussing with my mother in Turkey. For example, for my mother, the child has nothing, I label the child myself"*, aligning with the findings of [Habayeb et al. \(2020\)](#), who stated that grandparents can provide conflicting opinions and advice about the child's condition, which may influence the parents' relationship with the wider family, leading them to either hide the child's condition or decrease the amount of communication with them.

Initially, parents often denied or did not realise the autistic features, often normalising their child's behaviour as part of typical development, attributing it to a lack of awareness about autism, the absence of expert guidance, limited opportunities to compare their child with peers, and common beliefs, such as the idea that boys tend to speak later. Rather than suspecting autism, they often considered other explanations, including hearing impairment, speech and language delays, or attention-related difficulties. A mother expressed this uncertainty, saying, *"There was something, I felt something, but I didn't know what. Doesn't he hear? Is that why he's so grumpy?"* Concerns about hearing were particularly common. Parents frequently mentioned that hearing assessments were conducted during the diagnostic process, and audiology results were often included in reports shared by parents. This aligns with findings of [Sousa et al. \(2009\)](#) reported that 62 % of 54 autistic children initially raised concerns about hearing loss. Delays in speech and language development were also attributed by parents to factors such as bilingualism at home, limited family social interaction, or changes following the arrival of a sibling. As one mother reflected, *"He became a brother. He had a sister when he was 10–16 months old. I said, you know, maybe a situation of jealousy occurred."*

Families with prior knowledge about autism or child development, or those who have previously had a child diagnosed with autism, tended to recognise neurodiverse features and seek diagnosis earlier. Only one participant mentioned having prior knowledge about autism and child development, emphasising personal interest in the subject: *"I also knew a little bit about autism before. This is how I had read things about children, both necessary and unnecessary information."*

4.2.2. Acceptance

The arrival of an autistic child brings about a new and unfamiliar change in a parent's life. Access to appropriate services for the child involves the parents' acceptance and adaptation. Our findings indicated that parents' adaptation to the diagnosis aligned with the stages of grief outlined by [Kübler-Ross and Kessler \(2014\)](#). Parents reported beginning with shock, progressing to denial for some, and experiencing depression for others. A mother vividly expressed her initial feelings: *"It was very difficult. I could not accept it. I could not sleep. I cried whenever I saw him."* Over time, after a prolonged period of uncertainty, parents became more certain of their recognition of the situation and prepared themselves for the caregiving process. Additionally, responses to the diagnosis, such as gratitude, resignation, and viewing the situation as a test from a spiritual perspective, appeared to support their adaptation process. This fatalistic approach can serve as coping resources for families, offering a framework for meaning-making in the context of their child's diagnosis ([Heer et al., 2012; Meiza et al., 2019](#)).

Research suggests that fathers typically follow a different acceptance path than mothers ([Legg & Tickle, 2019](#)). One mother described the father's process as more subdued: *"His shock was a little quieter. I was more active, so I was doing something, crying, or searching. So, I was doing something"*. Two mothers emphasised that acceptance was primarily their responsibility, and mothers tended to be more solution-oriented, deeming it necessary for the benefit of the child. These gendered dynamics, however, were not always harmonious; differences in coping styles sometimes led to family conflict and, in more severe cases, marital breakdown. Prior research has linked fathers' quieter responses to feelings of exclusion during the assessment process, highlighting the need for more inclusive and gender-sensitive support ([Legg & Tickle, 2019](#)). While cultural factors were not explicitly discussed by our participants, our findings may also speak to broader cultural expectations around gender roles in parenting. As [Lashewicz et al. \(2019\)](#) argue, fathers from diverse cultural backgrounds often navigate additional layers of complexity; including language barriers, cultural norms, and stereotypes, which can influence how they experience and respond to a diagnosis of autism.

4.2.3. Parents' life experiences

The experiences of parents with autistic children highlight significant life changes and their impact. Many parents struggled to recall past experiences, focusing instead on the future - likely due to shock stages during adaptation, which can create 'shock memory.' One mother described how her child's autism reshaped every aspect of her life, leading to reprioritisation and reduced tolerance. Another parent emphasised the physical exhaustion from caregiving. Studies have shown that parental stress decreases when they are able to fulfil their parenting roles ([Hou et al., 2023](#)), underscoring the importance of addressing parental well-being.

The absence of extended family deprived parents of crucial social support in their daily lives, as explained by one of the mothers *"Their grandmother should look after them. Let me go and rest for a couple of hours and relax a bit. There is no such thing"*. While the culture of babysitting is growing in Türkiye and the Turkish communities ([İnan & Doğan-Temur, 2010](#)), the financial burden and the lack of trust in strangers often discourage parents ([Saritaş, 2020](#)). This reliance on parental care further isolated mothers, limiting their ability to work or engage socially. Particularly, single mothers faced overwhelming challenges, often feeling they had no life beyond caregiving, consistent with [Copeland and Harbaugh's \(2005\)](#) findings that single parents may experience high levels of anxiety and stress due to the lack of partner support.

Additionally, parents often felt a need to hide their child's condition from the broader community, leading to changes in their social relationships: *"They didn't know my child's diagnosis. I didn't want them to see that there was a difference in the child"*. This perceived stigma can contribute to social isolation, as families of autistic children may stay away from the community due to sensitivity to others' reactions ([Kearney & Griffin, 2001](#)). Despite maintaining relationships with other families of children with autism, parents may struggle to navigate social interactions outside of this circle.

4.3. Mesosystem - navigating the complexities of accessing support services

Findings concerning stakeholders beyond the parent, such as the school, peers, and immediate environment, were situated in the mesosystem, as they reflected parental interactions with these systems rather than the child's direct experiences, highlighting the crucial role of collaboration with others in accessing services.

4.3.1. Assessment experiences

Three parents encountered health issues during the prenatal period, leading to either pre-term or late births. Expectant mothers at risk of pre-term birth typically undergo intensive monitoring until delivery; however, this level of monitoring may not always be provided for post-term births. For example, one mother shared her experience of going through a late birth, detailing the lack of support until the bleeding began, and highlighted the negative psychological impact of such experiences on their mental health. A meta-analysis aimed to identify perinatal factors related to autism has indicated a correlation between pre-term birth and an increased likelihood of autism (Lavery et al., 2021). However, no established link has been found between post-term birth and autism. Nonetheless, pregnancy complications can significantly influence parental mental health and the child's developmental trajectories (Lahti et al., 2015). Thus, follow-up care could be essential to focus not only on the risk of pre-term birth but also on other complications.

Parents of autistic children can use various paths to seek assessment referrals, such as contacting their General Practitioner (GP), speech and language therapists, or charities. However, the application and referral process can be notably lengthy in the UK, with an average delay of approximately 3.5 years from the initial concern to the confirmation of autism (Crane et al., 2016). In Türkiye, assessments are typically conducted by child psychiatrists or paediatricians in public or private hospitals. The timeframe in Türkiye tends to be shorter than in the UK, between the initial concern and diagnosis, around 1–1.5 years (Yaylaci & Guller, 2023). As illustrated by one mother, the autistic features of their child were first noticed during a visit to Türkiye, where they had an initial assessment. However, upon returning to England, they faced delays in obtaining a diagnosis. Expressing frustration, the mother stated, *"[we] made an appointment with the GP. They said he was too young to be diagnosed and scheduled an appointment six months later. I did not want to wait and went back to Türkiye."*

4.3.2. Access to services

Parents of autistic children may require access to many different services based on their unique needs. This study's participants' experience of accessing the necessary services for themselves or their children were examined under three categories: educational, psychological and alternative services.

4.3.2.1. Educational services. The formal educational support received by the participants included various services, such as school attendance, group-based speech and language therapy, occupational therapy, and sensory integration therapy. Additionally, they benefited from one-to-one classroom support, family training, and charity support programs.

Some parents sought private educational services, such as one-to-one therapies at home or private institutions, including speech and language therapy and Applied Behaviour Analysis (ABA) therapy. Due to low expectations from school support, parents turned to home-based sessions to meet their children's basic educational needs. One mother expressed optimism about ABA, stating *"[name] is a kid who can move forward very well with ABA, because he is a child who likes working at the desk, with order and discipline."* However, recently, research and practice have increasingly shifted toward naturalistic approaches as behaviouristic methods and teaching socially desirable behaviours have become less accepted. Particularly within the neurodiversity movement, autistic advocates argue that such methods can undermine self-identity, encourage masking, and negatively affect mental health (Bernardin et al., 2021; Milton, 2018). Further research into how families perceive ABA therapy and why they seek private services would be valuable.

During the transition to school, children's individual differences become apparent, some adapt easily, while others need more time. Facilitating smooth transitions was a policy challenge (OECD, 2006), requiring collaboration between schools, preschools, families, and local communities (Hugo et al., 2018). One mother emphasised strong support: *"Because I don't speak much English, they knew that I could not do it [alone], they supported me a lot. Both the school pedagogue, an extra translator, and a teacher from the school came to school visits with me"*. In contrast, two mothers found that their schools were unwilling to cooperate, excluding families from the transition process and depriving their children of the education they deserved, highlighting the importance of schools building strong relationships and communication with parents, as highlighted by Falkmer et al. (2015). Some parents reported gaps in planning, affecting their children's placement options, *"In none of the annual reviews, not even the educational psychologist came and wrote a report again. The reports are still from the kindergarten. When we apply to another mainstream school with that report, they tell us this kid needs a lot of help; we can't get them"*, despite guidelines stating that a child's EHCPs should be reviewed annually with family participation (Department for Education and Department of Health and Social Care, 2015).

4.3.2.2. Psychological services. All parents emphasised the need for psychological support throughout the process. However, lacking guidance, some sought support independently, which they found to be crucial and helpful. Additionally, one mother expressed her intention to schedule an appointment with the NHS for this purpose. Yet, she felt pessimistic about the support she would receive due to difficulties in making appointments, long waiting times, and the language barrier.

4.3.2.3. Alternative services. Parents often explored alternative approaches such as diet modifications and supplements under a

consultant's supervision, particularly due to the lack of support for these practices within the UK healthcare system; they often obtain medical prescriptions and appointments in Türkiye. Harrington et al. (2006) noted that long waiting times for traditional healthcare services diminished parents' trust in healthcare professionals, leading them to seek alternative practices without scientific basis. This was reflected in researcher reflective journal, "*Many different alternative applications are discussed in social media groups. An active person in the group mentions conspiracy theories, stating that scientists should not be trusted. Parents respect her very much*" referring to an individual advocating for alternative practices.

4.4. Exosystem - Navigating legal regulations and official procedures

Legal regulations play a pivotal role in shaping service delivery and establishing the overall framework. While early intervention, defined as timely support to reduce the risk of poor outcomes and expected to begin as soon as developmental concerns arise (Dorset County Hospital NHS Foundation Trust, 2022), does not require a formal diagnosis and should be initiated through the graduated 'Assess, Plan, Do, Review' approach within early years settings or at home (SEND Code of Practice, 2015), participants reported receiving no such support prior to diagnosis.

Access to specialist services typically requires documented developmental delay or formal assessment. For instance, referrals to speech and language therapy are generally accepted after a period in which universal support strategies have not adequately met the child's needs (WSFT, 2025). As a result, delays in obtaining a diagnosis, often beyond age three due to long waiting times or eligibility thresholds, meant many families missed out on both early and specialist support during a critical developmental period. Indeed, research by Crane et al. (2016) and Smith-Young et al. (2020) underscored the prolonged period between parents first noticing their child may need support and accessing the relevant services. One mother described this situation as a "war", highlighting the challenge of accessing appropriate services. This gap between policy and practice represents a substantial barrier to timely intervention, particularly in the 0–3-year period, which is widely recognised as crucial for improving parent–child interaction and developmental trajectories (Green et al., 2017).

Moreover, one mother reported that while a teaching assistant was assigned to support her child, the assistant's attention was divided due to the needs of many other students. Despite the SEND Code of Practice (2015) requiring schools to provide support for every child in need, the mother felt there was insufficient help for both her child and others. Although second- and third-level support can be offered based on individual needs, challenges may arise in accurately assessing and meeting those needs during implementation.

4.5. Macrosystem - Cultural perspectives and coping strategies

Culture, perception and belief systems shape individuals' attitudes and behaviours, and in this study, the experiences of immigrant parents were examined through the lens of how their cultural frameworks and diverse backgrounds influenced their interpretations and daily lives.

4.5.1. Conceptualisation of autism

When discussing the concept of autism with the participants, four parents referred to it as a "disease," and one defined it as "a child who cannot express himself." Another mother described autism within the framework of "normality" due to her child not exhibiting any physical and cognitive differences. These interpretations may reflect the influence of ableism, which can be defined as a system of beliefs and practices that position non-disabled bodies and minds as inherently more valuable, often reinforcing social hierarchies based on ability (More, 2023). In the Turkish context, ableism operates through internalised social norms that idealise physical and cognitive "normality," often marginalising those who deviate from these standards (Ünal, 2018). This cultural framing may shape how families understand autism; not as a form of neurodiversity, but as a deviation from what is socially accepted.

Beliefs are central components in informing a parent's attitude and approach to their child's condition. Regarding those core beliefs, one mother explained that her child "*is not in a position to understand evil. How happy for you! Just think about it: a living thing you give birth to that will not harm the world.*" Kennett (2002) refers to this type of belief as an expression of "attractive moral innocence", a framing in which the child is seen as embodying a kind of moral purity, untainted by deception, cruelty, or malice. Although this innocence is distinguished from full moral agency, the belief itself can be deeply meaningful for parents and may offer a sense of reassurance. Additionally, some believed that a child's disability was a punishment from God for sins committed. This notion is primarily rooted in belief systems and is consistent with findings from a study conducted by Treloar (2002).

4.5.2. The influence of cultural and linguistic diversity

The influence of culturally and linguistically diverse backgrounds was evident in various aspects of supporting the development of the child, family relationships, and interactions with other stakeholders and society at large.

Parents generally found those they interacted with to be friendly and accepting of autism, with high levels of communication and social acceptance. However, they often doubted the sincerity of some individuals due to misinformation from teachers about entitled services. Additionally, the facial expressions of those they discussed their child's condition with sometimes revealed a simplified understanding of the situation: "*When something happens to someone else's child, our eyes fill with tears. But here, everything is minimised. Like, 'Your child is autistic, just accept it,' with a smile, as if it's nothing serious.*"

Parents utilised the advantage of their diverse cultural backgrounds and language abilities to access support services and resources both in their current and home countries. However, language barriers remained significant despite interpreter support, leading to the

need for intermediaries in communication. The [EMTAS Project \(2011\)](#) highlighted another consequence of the language barrier: experts, teachers, and parents often postpone the application or referral process due to lower progress in the child's language development than their peers. The complex layers of the intersection of having a neurodiverse child and speaking another language have been exemplified by some parents. They stopped speaking their mother tongue with their children out of fear of confusing them, despite recommendations to continue its use: *"If other families are bilingual, they shouldn't give up their language, because I can see now that my son actually had that capacity. He could learn two languages... I want to return to [using] Turkish."*

4.6. Chronosystem - Changes over time

The Chronosystem reflects changes in situations and mentalities over time. Participants reported initially feeling alienated by their circumstances, with a mother describing an inability to enjoy time with their child at the beginning. However, this perception changed over time: *"When they said that ... you will have a lot of fun when spending time with him, I said to my partner, 'Look at these people: cold comfort', [but] I am having fun with [name] right now. I'm really having fun ... But this was a ridiculous thought for me two years ago."*

Furthermore, participants described a shift in their perception of their child as their development progressed. As they learned effective interaction and communication strategies, their understanding deepened. A father highlighted the need for adaptation due to cultural and temporal shifts in child-rearing practices. The arrival of an autistic child prompted a lifestyle adjustment, initially presenting an unfamiliar situation. However, over time, they found a new balance in their lives.

5. Limitations and recommendations

This study focused on first-generation immigrant parents from Türkiye, which limits insight into how experiences and beliefs may shift across generations. Future research could include second- and third-generation parents to explore whether cultural interpretations of autism, such as viewing it as destiny or through an ableist lens, persist over time. Also, longitudinal qualitative studies would help to understand how parental decisions and interpretations evolve and how these shape children's long-term outcomes.

The study primarily examined cultural background, with less attention to intersecting factors. Future work should consider variables such as socioeconomic status, language proficiency, and discrimination to better understand the layered experiences of immigrant families.

When presenting the findings, direct quotations have been translated from Turkish to English for the benefit of readers. To ensure accuracy, these translations were then back-translated into Turkish, and at each stage, the translations were reviewed and confirmed by a native speaker of the respective language. However, participants and first language readers might prefer to read in Turkish due to potential differences in expressions and nuances best captured in their first language. In future publications intended for an international audience, including bilingual quotations can be considered.

6. Conclusion

This study explored how first-generation Turkish immigrant parents navigated early support services from the stage of initial concerns through the education process. Early signs were often misinterpreted or downplayed, shaped by cultural expectations, limited awareness, and delayed professional input. Mothers typically led the emotional and practical adaptation in the absence of extended family support. As they progressed, parents developed coping strategies and sought meaning through spiritual beliefs and deepening emotional connection, highlighting the need for early, culturally responsive support throughout the diagnostic and educational journey.

CRediT authorship contribution statement

Paulette Luff: Supervision. **Gamze Kaplan:** Writing – review & editing, Writing – original draft, Methodology, Formal analysis, Data curation, Conceptualization.

Declaration of Competing Interest

The authors declare the following financial interests/personal relationships which may be considered as potential competing interests: Gamze Kaplan reports financial support was provided by The Republic of Türkiye Ministry of National Education. If there are other authors, they declare that they have no known competing financial interests or personal relationships that could have appeared to influence the work reported in this paper.

Acknowledgements

This work was supported by the [blinded to maintain the integrity of the review process].

Data availability

The authors do not have permission to share data.

References

- APA. (2013). *Diagnostic and statistical manual of mental disorders (5th ed.)* (American Psychiatric Association). American Psychiatric Publishing.
- Baykusoglu, S. (2014). The role of parents in the achievement of Turkish-speaking students in British schools: Why they fail in supporting their children? *US-China Education Review*, 4(10), 688–704.
- Bernardin, C. J., Mason, E., Lewis, T., & Kanne, S. (2021). You must become a chameleon to survive”: Adolescent experiences of camouflaging. *Journal of Autism and Developmental Disorders*, 51, 4422–4435. <https://doi.org/10.1007/s10803-021-04912-1>
- Bronfenbrenner, U. (1979). *The ecology of human development: Experiments by nature and design*. Harvard University Press.
- Bronfenbrenner, U., & Morris, P. A. (2006). The bioecological model of human development. In W. Damon, & R. Lerner (Eds.), *Handbook of Child Psychology: Theoretical Models Of Human Development* (pp. 793–828). John Wiley & Sons Inc.
- Copeland, D. B., & Harbaugh, B. L. (2005). Differences in parenting stress between married and single first-time mothers at six to eight weeks after birth. *Issues in Comprehensive Pediatric Nursing*, 28(3), 139–152. <https://doi.org/10.1080/01460860500227556>
- Crane, L., Chester, J. W., Goddard, L., Henry, L. A., & Hill, E. (2016). Experiences of autism diagnosis: A survey of over 1000 parents in the United Kingdom. *Autism*, 20(2), 153–162. <https://doi.org/10.1177/1362361315573636>
- Department for Education & Department of Health and Social Care (2015). *Special educational needs and disability code of practice: 0–25 years* [online]. (https://assets.publishing.service.gov.uk/media/5a7dcb85ed915d2ac884d995/SEND_Code_of_Practice_January_2015.pdf).
- Dorset County Hospital NHS Foundation Trust (2022). Development and behaviour pathway: Early years / preschool (0-5 years). NHS. (<https://www.dchft.nhs.uk/wp-content/uploads/2022/09/Development-pathway-Early-years-0-5-Final.pdf>).
- Essex County Council, 2011. Supporting Thai, Turkish & Turkish Cypriot pupils at key stages 1–4. [online] <https://schools.essex.gov.uk/pupils/EMTAS%20Ethnic%20Minority%20and%20Traveller%20Achievement%20Service/EAL/Documents/Findings%20from%20the%20Thai%20and%20Turkish%20Project.pdf>.
- Falkmer, M., Anderson, K., Joosten, A., & Falkmer, T. (2015). Parents’ perspectives on inclusive schools for children with autism spectrum conditions. *International Journal of Disability Development and Education*, 62(1), 1–23. <https://doi.org/10.1080/1034912X.2014.984589>
- Feng, Y., Zhou, X., Qin, X., Cai, G., Lin, G., Pang, Y., Chen, B., Deng, T., & Zang, L. (2022). Parental self-efficacy and family quality of life in parents of children with autism spectrum disorder in China: The possible mediating role of social support. *Journal of Pediatric Nursing*, 63, 159–167. <https://doi.org/10.1016/j.pedn.2021.10.014>
- Green, J., Pickles, A., Pasco, G., Bedford, R., Wan, M. W., Elsabbagh, M., Slonims, V., Gliga, T., Jones, E., Cheung, C., Charman, T., Johnson, M., & The British Autism Study of Infant Siblings (BASIS) Team. (2017). Randomized trial of a parent-mediated intervention for infants at high risk for autism: Longitudinal outcomes to age 3 years. *Journal of Child Psychology and Psychiatry*, 58(12), 1330–1340. <https://doi.org/10.1111/jcpp.12728>
- Habayeb, S., Dababnah, S., John, A., & Rich, B. (2020). Cultural experiences of Arab American caregivers raising children with autism spectrum disorder. *Journal of Autism and Developmental Disorders*, 50(1), 51–62. <https://doi.org/10.1007/s10803-019-04218-3>
- Harrington, J. W., Patrick, P. A., Edwards, K. S., & Brand, D. A. (2006). Parental beliefs about autism: Implications for the treating physician. *Autism*, 10(5), 452–462. <https://doi.org/10.1177/1362361306066609>
- Heer, K., Larkin, M., Burchess, I., & Rose, J. (2012). The cultural context of care-giving: qualitative accounts from South Asian parents who care for a child with intellectual disabilities in the UK. *Advances in Mental Health and Intellectual Disabilities*, 6(4), 179–191. <https://doi.org/10.1108/20441281211236580>
- Heidegger, M. (1962). *Being and time*. Translated from German by J. Macquarrie and E. Robinson. Harper-Collins Publishers.
- Hou, Y., Yan, T., & Zhang, J. (2023). The relationship between parental involvement and psychological adjustment among Chinese children with autism spectrum disorder in the transition from kindergarten to primary school: a chain-mediating model. *Frontiers in Psychology*, 14. <https://doi.org/10.3389/fpsyg.2023.1087729>
- House of Commons Home Affairs Committee, 2011. Implications for the justice and home affairs area of the accession of Turkey to European Union: Tenth report of session 2010–12. [online] (<https://www.statewatch.org/media/documents/news/2011/aug/eu-hasc-turkey-jha-report.pdf>).
- Hugo, K., McNamara, K., Sheldon, K., Moul, F., Lawrence, K., Forbes, C., Martin, N., & Miller, M. G. (2018). Developing a blueprint for action on the transition to school: Implementation of an action research project within a preschool community. *International Journal of Early Childhood*, 50, 241–257. <https://doi.org/10.1007/s13158-018-0220-1>
- Husserl, E. (1931). *Ideas*. Translated from German by W. R. B. Gibson, 2012. Routledge Classics.
- Information NOW (2024). *Grandparent issues*. Newcastle City Council. (<https://www.informationnow.org.uk/article/grandparent-issues/#>).
- İnan, H. Z., & Doğan-Temur, Ö. (2010). Babysitters in early childhood: A research on parents’ experiences and perspectives on babysitters. *Inonu University Journal of the Faculty of Education*, 11(2), 1–17.
- Kaplan, G., & Celik, P. (2023). Intersectionality of disability and cultural/linguistic diversity in the UK: a literature review. *Frontiers in Education*, 8(1239777), 1–13. <https://doi.org/10.3389/educ.2023.1239777>
- Kearney, P. M., & Griffin, T. (2001). Between joy and sorrow: Being a parent of a child with developmental disability. *Journal of Advanced Nursing*, 34(5), 582–592. <https://doi.org/10.1046/j.1365-2648.2001.01787.x>
- Kennett, J. (2002). Autism, empathy and moral agency. *The Philosophical Quarterly*, 52(208), 340–357. <https://doi.org/10.1111/1467-9213.00272>
- Kubler-Ross, E., & Kessler, D. (2014). *On grief & grieving: Finding the meaning of grief through the five stages of loss*. Simon & Schuster.
- Lahti, M., Eriksson, J. G., Heinonen, K., Kajantie, E., Lahti, J., Wahlbeck, K., Tuovinen, S., Pesonen, A. K., Mikkonen, M., Osmond, C., Barker, D. J. P., & Räikkönen, K. (2015). Late preterm birth, post-term birth, and abnormal fetal growth as risk factors for severe mental disorders from early to late adulthood. *Psychological Medicine*, 45(5), 985–999. <https://doi.org/10.1017/S0033291714001998>
- Lashewicz, B. M., Shipton, L., & Lien, K. (2019). Meta-synthesis of fathers’ experiences raising children on the autism spectrum. *Journal of Intellectual Disabilities*, 23(1), 117–131. <https://doi.org/10.1177/1744629517719347>
- Laverty, C., Surtees, A., O’Sullivan, R., Sutherland, D., Jones, C., & Richards, C. (2021). The prevalence and profile of autism in individuals born preterm: a systematic review and meta-analysis. *Journal of Neurodevelopmental Disorders*, 13(41), 1–12. <https://doi.org/10.1186/s11689-021-09382-1>
- Legg, H., & Tickle, A. (2019). UK parents’ experiences of their child receiving a diagnosis of autism spectrum disorder: A systematic review of the qualitative evidence. *Autism*, 23(8), 1897–1910. <https://doi.org/10.1177/1362361319841488>
- Li, F., Xu, M., Wu, D., Tang, Y., Zhang, L., Liu, X., Zhou, L., Li, F., & Jiang, L. (2022). From child social impairment to parenting stress in mothers of children with ASD: The role of parental self-efficacy and social support. *Frontiers in Psychiatry*, 13, 1–9. <https://doi.org/10.3389/fpsyg.2022.1005748>
- Marsack-Topolewski, C. N., & Weisz, A. N. (2020). Parents’ perceptions of access to services for their adult children diagnosed with autism spectrum disorder. *Families in Society The Journal of Contemporary Social Services*, 101(2), 190–204. <https://doi.org/10.1177/10443894198812>
- Meiza, A., Kardinah, N., Rahman, A. A., & Puspasari, D. (2019). Quantitative profile of family acceptance of children special need’s Moslem parents (Case study at Rumah Terapi Aura). *The American Journal of Family Therapy*, 47(4), 232–243. <https://doi.org/10.1080/01926187.2019.1635540>
- Milton, D. (2018). A critique of the use of Applied Behavioural Analysis (ABA): on behalf of the Labour Party Autism / Neurodiversity Manifesto Steering Group. Appendix to the Labour Party Autism / Neurodiversity Manifesto. (<https://neurodiversitymanifesto.com/2018/09/18/labour-party-autism-neurodiversity-manifesto-final-draft-version-2018/>).
- More, R. (2023). Storying ableism: proposing a feminist intersectional approach to linking theory and digital activism. *Feminist Theory*, 25(3), 322–337. <https://doi.org/10.1177/14647001231173242>
- National Autistic Society, 2020. Post-diagnosis support. [online] (<https://www.autism.org.uk/advice-and-guidance/topics/diagnosis/post-diagnosis-support/parents-and-carers/>).
- NHS, National Health Services, 2019a. How to get diagnosed. [online] (<https://www.nhs.uk/conditions/autism/getting-diagnosed/how-to-get-diagnosed/>).
- NHS, National Health Services, 2019b. Newly diagnosed things to help. [online] (<https://www.nhs.uk/conditions/autism/newly-diagnosed/>).
- OECD (2006). Starting strong II: Early childhood education and care [online]. (<https://www.oecd.org/education/school/startingstrongearlychildhoodeducationandcare.htm#HTO>).

- Sandhu, P., Ibrahim, J., & Chinn, D. (2017). 'I wanted to come here because of my child': Stories of migration told by Turkish-speaking families who have a son or daughter with intellectual disabilities. *Journal of Applied Research in Intellectual Disabilities*, 30(2), 371–382. <https://doi.org/10.1111/jar.12248>
- Sarıtaş, C. T. (2020). Precarious contours of work–family conflict: The case of nurses in Turkey. *The Economic and Labour Relations Review*, 31(1), 59–75. <https://doi.org/10.1177/1035304619879327>
- Sicherman, N., Loewenstein, G., Tavassoli, T., & Buxbaum, J. D. (2018). Grandma knows best: Family structure and age of diagnosis of autism spectrum disorder. *Autism*, 22(3), 368–376. <https://doi.org/10.1177/1362361316679632>
- Smith, J. A., Flowers, P., & Larkin, P. (2009). *Interpretative phenomenological analysis: Theory, method and research*. Sage.
- Smith-Young, J., Chafe, R., & Audas, R. (2020). "Managing the wait": parents' experiences in accessing diagnostic and treatment services for children and adolescents diagnosed with autism spectrum disorder. *Health Services Insights*, 13. <https://doi.org/10.1177/1178632920902141>
- Souralova, A. (2020). Grandparents, kinship ties, and belonging after migration: the perspective of second-generation grandchildren. *Global Networks*, 20(2), 362–379. <https://doi.org/10.1111/glob.12240>
- Sousa, E. C., Lima, F. T., Tamanaha, A. C., Perissinoto, J., de, Azevedo, M. F., & Chiari, B. M. (2009). A associação entre a suspeita inicial de perda auditiva e a ausência de comunicação verbal em crianças com transtornos do espectro autístico. *Revista da Sociedade Brasileira Délelôtt Fonoaudiologia*, 14(4), 487–490. <https://doi.org/10.1590/S1516-80342009000400010>
- Spier, J., & Smith, J. A. (2019). Interpretative phenomenological analysis. In P. Atkinson, S. Delamont, A. Cernat, J. W. Sakshaug, & R. A. Williams (Eds.), *Foundations. Sage Research Methods*. <https://doi.org/10.4135/9781526421036813346>
- Taylor, S. J., Bogdan, R., & DeVault, M. (2016). *Introduction to qualitative research methods: A guidebook and resource*. Wiley.
- The Children Act 2004. (<https://www.legislation.gov.uk/ukpga/2004/31/contents>).
- Treloar, L. L. (2002). Disability, spiritual beliefs and the church: the experiences of adults with disabilities and family members. *Journal of Advanced Nursing*, 40(5), 594–603. <https://doi.org/10.1046/j.1365-2648.2002.02417.x>
- Ünal, B. (2018). The development of disability pride through challenging internalized idealist and ableist norms in Turkish society: A grounded theory study [Doctoral dissertation, Middle East Technical University]. (<https://open.metu.edu.tr/handle/11511/27095>).
- Uysal, A. (2016). Londra'daki Türk nüfusun mekânsal dağılımı. *Marmara Coğrafya Dergisi*, 0(33), 534–565. <https://doi.org/10.14781/mcd.61591>
- van Manen, M. (2014). *Phenomenology of practice: Meaning-giving methods in phenomenological research and writing*. Left Coast Press.
- van Manen, M. (2017). But is it phenomenology? *Qualitative Health Research*, 27, 775–779. <https://doi.org/10.1177/1049732317699570>
- Varol, N. (2006). *Aile eğitimi*. 2nd ed. Kök Yayıncılık.
- WSFT, West Suffolk NHS Foundation Trust (2025). Paediatric speech and language therapy service specific guidelines for referrers. NHS. (<https://www.wsh.nhs.uk/CMS-Documents/ICPS/SLT-Guidance-for-Referrers.pdf>).
- Webb, S. J., & Jones, E. J. (2009). Early identification of autism: Early characteristics, onset of symptoms, and diagnostic stability. *Infants & Young Children*, 22(2), 100–118. <https://doi.org/10.1097/YYC.0b013e3181a02f7f>
- Yaylaci, F., & Guller, B. (2023). Potential barriers to early diagnosis of autism in Turkey: an evaluative study. *Children's Health Care*, 1–26. <https://doi.org/10.1080/02739615.2023.2232723>
- Yin, R. K. (2013). *Case Study Research: Design and Methods* (5th ed.). Sage Publication.
- Zahavi, D. (2019). Getting it quite wrong: Van Manen and Smith on phenomenology. *Qualitative Health Research*, 29(6), 900–907. <https://doi.org/10.1177/104973231881754>